PRINTABLE BUNDLE · 33 PAGES

FOR ADULT CHILDREN OF AGING PARENTS

The Caregiver Binder

Three kits, in the order life hands them to you: the holiday visit where you first notice, the week home after a hospital stay, and the first month of caregiving. Print the pages you need, when you need them.

PART 1 · 8 PAGES

Home for the Holidays: Is It Time?

36 signs, the kitchen walk, three conversations, the family debrief, green / yellow / red.

PART 2 · 11 PAGES

After the Hospital: The First 7 Days Home

Before you leave, medication changes, when to call, the 7-day plan, help roster.

PART 3 · 14 PAGES

The First 30 Days

Roadmap, fridge sheet, medication list and log, doctor questions, documents, caregiver check-in.

PRINTABLE KIT · 8 PAGES

BEFORE YOU VISIT YOUR PARENTS THIS SEASON

Home for the Holidays: Is It Time?

A quiet way to notice whether a parent needs more help, three conversations that don't start a fight, and a plan for the drive home.

WHAT'S INSIDE

- How to use this weekend
- 36 signs across six areas
- The kitchen walk
- Three conversations that work
- What not to say
- The family debrief
- Green, yellow, red: what to do next
- Next steps tracker

START HERE

How to use this weekend

You're not there to inspect. You're there for the holiday. This kit lets you notice things while you're carving the turkey, and gives you words for the conversation if you need them.

Before you go

- [ ] Read the 36 signs once so they're in the back of your mind
- [ ] Pick one sibling or relative to compare notes with afterward
- [ ] Decide: this visit is for noticing, not fixing
- [ ] Print the debrief page; keep it in the car

While you're there

- [ ] Offer to cook or clean up. The kitchen tells you the most.
- [ ] Ride along somewhere if they drive
- [ ] Look at the mail pile and the pill bottles without commenting
- [ ] Notice how they move from chair to kitchen to bathroom
- [ ] Mark the signs page privately, later, not in front of them

The three rules

1. One thing at a time. If you see twelve problems, you still raise one. The conversation you want to have is the first of several, not the only one.

2. Curious, not corrective. "I noticed the stairs seemed harder this year" opens a door. "You shouldn't be using those stairs" closes it. Scripts on page 5.

3. Their life, their say. Unless safety is immediate, the goal is to be invited in, not to take over. Parents who feel managed start hiding things.

If you see something that can't wait

A burn or fall they didn't mention, spoiled food they're eating, medications badly mixed up, a car with new dents, confusion about who you are. Skip to page 7. Those are red, and red means this week, not after New Year's.

MARK PRIVATELY, AFTER

36 signs, six areas

Fine · Not sure / small change · Noticed clearly

The house

Upkeep slips before anything else.

- [ ] Clutter or mail piling up where it didn't before
- [ ] Dishes, laundry, or bathroom noticeably less clean
- [ ] Burned pots, scorch marks, a smoke detector with no battery
- [ ] Lights out, repairs undone, yard let go
- [ ] Rugs, cords, or stacks in the walking path
- [ ] Pets or plants not cared for the way they were

The kitchen

Food is the most honest record of a week.

- [ ] Expired or spoiled food in the fridge
- [ ] Very little fresh food; mostly cans, crackers, sweets
- [ ] Same few meals, or skipping meals
- [ ] Weight loss you can see in their clothes or face
- [ ] Multiples of the same item bought (forgot they had it)
- [ ] Stove or appliances they've stopped using

Their body

Watch how they move, not what they say.

- [ ] Holding furniture or walls to get around
- [ ] Bruises, especially arms and shins, or a fall they minimized
- [ ] Slower on stairs, or avoiding them
- [ ] Trouble hearing the conversation, or TV very loud
- [ ] Grooming slipping: hair, nails, shaving, same clothes
- [ ] Shortness of breath or new unsteadiness

Mind & mood

Repetition and withdrawal are the early tells.

- [ ] Repeating the same story or question in one visit
- [ ] Losing the thread of a conversation or a recipe they know
- [ ] Mixing up names, dates, or recent events
- [ ] Irritable, suspicious, or flat in a way that's new
- [ ] Dropped hobbies, church, friends, or calls
- [ ] Sleeping much more, or up at odd hours

Money & mail

Where exploitation shows up first.

- [ ] Unopened bills, or late notices
- [ ] Unusual purchases, subscriptions, or charity mailings
- [ ] Checks written to unfamiliar names
- [ ] Talk of a new "friend," prize, or investment
- [ ] Confusion about what's been paid
- [ ] Unusually guarded about money questions

Driving & out and about

Ask to ride along. Don't lecture in the car.

- [ ] New dents, scrapes, or a mirror knocked off
- [ ] Drifting, slow reactions, missed signs
- [ ] Getting lost on a familiar route
- [ ] Driving only in daylight, or avoiding it entirely
- [ ] Missed appointments or stopped going places
- [ ] Traffic tickets or a warning they mentioned in passing

WHILE YOU COOK OR CLEAN UP

The kitchen walk

Offer to make coffee, load the dishwasher, put leftovers away. Ten minutes in the kitchen tells you more than an hour of asking "how are you doing, really?"

LOOK AT	WHAT YOU'RE CHECKING	WHAT YOU SAW
Fridge	Dates on milk, meat, leftovers. Fresh produce present? Same containers as last visit?	
Pantry	Duplicates (three of the same jar). Mostly shelf-stable, sugary, or easy-open food?	
Stove & microwave	Scorched pans, burnt smell, splatter left for weeks, knobs they avoid.	
Counter	Pill bottles: how many, any from the same drug twice, expired dates, a pill organizer with wrong days filled.	
Mail pile	Unopened envelopes, "final notice," sweepstakes or charity mail in volume.	
Trash	Lots of takeout, or almost nothing (not eating). Broken glass or evidence of a spill nobody mentioned.	
Floor & lighting	Sticky spots, crumbs, dim bulbs, a rug that slides.	
Them, in the kitchen	Reaching high shelves, opening jars, reading labels, remembering where things go.	

Other rooms, if it's natural

Bathroom: grab bars, a slippery tub, towels, how clean. Bedroom: clear path to the bathroom, lamp within reach. Entry: can they manage the steps and the locks?

Notes

ONLY IF YOU NEED THEM

Three conversations that work

Pick one. Have it one-on-one, not at the table, not on the last morning. Say your line, then stop talking. The silence is where they answer.

1 · THE "I NOTICED" OPENER · FOR ONE SPECIFIC THING

"Mom, I noticed the stairs seemed harder this year. How are they feeling to you?"

"Dad, I saw a couple of bills on the counter from October. Want me to sit with you and go through the pile before I leave?"

Why it works: one observation, no diagnosis, a question they can answer honestly. If they brush it off, you've still planted it. Let it go for now.

2 · THE FUTURE FRAMING · FOR THE BIGGER PICTURE

"You're doing fine right now. I'm asking about later. If a day came when you needed more help at home, what would you want that to look like? Who would you want involved?"

"I'd rather know what you want while you can tell me, than guess in a hospital hallway."

Why it works: it's about their wishes, not your worries. It opens the door to paperwork (power of attorney, directives) without making it about decline.

3 · THE ASK FOR HELP · WHEN YOU NEED THEM TO ACCEPT SOMETHING

"It would honestly take a weight off me if you'd let someone come by once a week to help with the house. Would you do that for me, even if you don't think you need it?"

"Would you be willing to see Dr. ___ in January and let me come along? I'd just like to hear it from her."

Why it works: parents will do for you what they won't do for themselves. A specific, small, time-limited ask beats a general one.

What not to say

"You can't live like this." "We've decided." "You're not safe here anymore." "Remember, I told you that yesterday." Anything that starts with "You always" or "You never." Anything said in front of the grandkids.

If they get angry

"Okay. I hear you. I'm not going to push today." Then don't. You can come back to it by phone in two weeks. One hard conversation that ends calmly is worth more than winning it.

IN THE CAR, OR ON THE PHONE THAT WEEK

The family debrief

Siblings see different things and remember them differently. Compare notes within a week, while it's fresh, and agree on three next steps with names on them. Fill one copy per person, or one together.

WHO	WHAT I NOTICED (THE FACTS, NOT THE WORRY)	GREEN / YELLOW / RED

Where we agree

Where we see it differently

Three next steps, with names

STEP	WHO	BY WHEN
1.		
2.		
3.		

Things worth deciding now

- [ ] Who is the point person for Mom/Dad
- [ ] Who talks to the doctor
- [ ] Who looks at the money, if anyone
- [ ] How often we check in with each other
- [ ] What we tell Mom/Dad we're doing
- [ ] When we revisit this (date)

DECISION GUIDE

Green, yellow, red: what to do next

Count your marks from the signs page. The color isn't a diagnosis. It's how fast to move.

Green · Mostly fine, a few small changes

- [ ] Say nothing heavy. Enjoy the visit.
- [ ] Write down what you saw and the date; it's your baseline for next time
- [ ] Have conversation #2 (the future framing) sometime this year, not this weekend
- [ ] Make sure someone knows where their important papers are
- [ ] Call more often. Weekly beats quarterly for catching change.

Yellow · Several "not sure" marks, or one area clearly slipping

- [ ] Have one conversation (#1 or #3), about one thing
- [ ] Ask for a doctor's visit in the next 60 days and offer to come along; many changes have treatable causes (medications, hearing, vision, thyroid, depression, infections)
- [ ] Set up the basics: a current medication list and an emergency information sheet on the fridge
- [ ] Light help first: a cleaner every two weeks, grocery delivery, a neighbor who checks in
- [ ] Debrief with family and set a date to look again, 60 to 90 days out

Yellow is where our companion kit, *The First 30 Days*, picks up: the fridge sheet, medication list, questions for the doctor, documents checklist, and a month-long plan.

Red · Safety, health, or money is at risk now

- [ ] Falls or injuries they hid; burns; spoiled food being eaten; medications badly mixed up; confusion about who you are; a car with fresh damage; money going to a stranger
- [ ] This week: call their doctor's office, describe what you saw, ask for an urgent appointment or a home health evaluation
- [ ] Today, if it's acute (sudden confusion, a fall with pain, chest or breathing trouble): urgent care or 911
- [ ] Someone stays, or someone comes daily, until there's a plan
- [ ] Driving: if you'd be afraid to ride with them, don't let the grandkids. Say so plainly and take the keys conversation to the doctor.
- [ ] Money: freeze what you can, call the bank's elder-fraud line, and the Area Agency on Aging for Adult Protective Services guidance

AFTER THE VISIT

Next steps tracker

What you decided, who owns it, and what happened. Keep it with the debrief page and bring both to the next visit.

Visit date: _____ Overall: ☐ Green ☐ Yellow ☐ Red Look again on: _____

DATE	STEP	WHO	OUTCOME	FOLLOW-UP

What I want to remember about this visit

Questions for the doctor, if we get there

This kit helps you notice and talk. It is not medical, legal, or financial advice, and a checklist can't diagnose anything. Changes you notice deserve a conversation with your parent's doctor, who can tell the difference between normal aging and something treatable.

PRINTABLE KIT · 11 PAGES

FOR THE WEEK THAT DECIDES WHETHER THEY GO BACK

After the Hospital: The First 7 Days Home

What to get before you leave the hospital, how to sort the new medications from the old, what to watch for each day, and who's covering which shift.

WHAT'S INSIDE

- Before you leave the hospital
- Medication changes sheet
- The 7-day plan
- When to call, when to go
- Follow-ups & services tracker
- Daily vitals & symptom log
- Setting up the house
- The help roster
- Questions for the follow-up visit
- Call log

DISCHARGE DAY · IN THE HOSPITAL ROOM

Before you leave the hospital

Discharge happens fast, usually when you're both tired. Most of what goes wrong in the first week traces back to something that wasn't asked here. Check these off before the wheelchair arrives.

Papers in hand

- [ ] Discharge summary, printed
  Diagnosis, what was done, what's next, in writing.
- [ ] The new medication list, printed
  Copy it onto page 3 with the nurse while they're in the room.
- [ ] Written care instructions
  Wound care, diet, activity limits, bathing, lifting.
- [ ] Follow-up appointments, with dates
  If they say "call to schedule," ask them to book it now.
- [ ] Prescriptions sent to the pharmacy, confirmed
  Ask which ones are new and which replace something.
- [ ] The after-hours number for the team that treated them

Questions for the discharge nurse

- [ ] What should I watch for, and what exactly do I do if I see it?
- [ ] Which symptoms mean call you, and which mean call 911?
- [ ] What was stopped, changed, or added on the medication list, and why?
- [ ] Can they shower? Climb stairs? Drive? Be alone? For how long?
- [ ] What equipment do they need at home, and who is delivering it?
- [ ] Is home health, physical therapy, or a visiting nurse ordered? Who calls whom?
- [ ] What's the diet, in plain words?
- [ ] When is the next appointment, and what should we bring?

Before you drive away

- [ ] Pharmacy has the prescriptions and they're ready, or you know when
- [ ] Equipment (walker, oxygen, commode, bed) is home or arriving today
- [ ] Someone is staying the first night
- [ ] Fridge has food that matches the diet
- [ ] Pathway from car to bed is clear
- [ ] Page 4 of this kit is filled in from the discharge papers

This kit organizes the instructions your care team gives you. It does not replace them. When this kit and your discharge papers disagree, the discharge papers win, and the after-hours number is the tiebreaker.

DISCHARGE DAY · FILL IN WITH THE NURSE, CONFIRM WITH THE PHARMACIST

Medication changes sheet

Readmissions often start with two lists in the house: the old bottles in the cabinet and the new ones from the pharmacy. This page is the one list. Then clear the old bottles out.

MEDICATION & STRENGTH	STATUS	DOSE & WHEN	WHAT IT'S FOR	NOTES (REPLACES WHAT? WITH FOOD? STOP DATE?)
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			

Three questions for the pharmacist

- ☐ Is anything on this list a duplicate of something they already take under another name?
- ☐ Does anything here interact with their old medications or supplements?
- ☐ Which of these can be stopped on a date, and

Old bottles removed from the house (name, date)

FILL IN FROM THE DISCHARGE PAPERS · POST IT NEXT TO THE FRIDGE SHEET

When to call, when to go

Your discharge papers list the warning signs for this condition. Copy them into the boxes below so nobody has to find the paperwork at 2 a.m. The general signs printed here apply to almost every discharge.

Call 911 now

Chest pain or pressure · trouble breathing · face drooping, arm weakness, slurred speech · sudden severe confusion or can't wake them · heavy bleeding that won't stop · a fall with a head strike, or they can't get up · fever with shaking chills after surgery

Condition-specific, from the discharge papers:

Call the care team today (use the after-hours number if needed)

Fever · redness, swelling, warmth, or drainage at an incision or wound · new or worse pain not controlled by what was prescribed · not keeping food or fluids down · no urine for many hours, or burning · new swelling in a leg · new confusion, dizziness, or a near-fall · a missed or doubled medication you're unsure about

Condition-specific, from the discharge papers:

Watch and note (bring to the follow-up)

Appetite, sleep, energy, mood, bathroom changes, mild soreness that's improving. Write it on the daily log; it's what the doctor will ask about.

WHO	NUMBER	WHEN TO USE IT
Care team after-hours line		Nights, weekends, anything on the yellow list
Primary care doctor		Follow-up, refills, questions in office hours
Pharmacy		Medication questions, missed doses
Home health / visiting nurse		Wound checks, vitals, therapy

THE WEEK

The 7-day plan

A default rhythm for the week, to adjust to your discharge instructions. Check boxes as you go. The daily log on page 7 holds the numbers.

Day 1
Home day

- [ ] Medication changes sheet done, old bottles out
- [ ] Pill organizer filled for the week
- [ ] "When to call" page filled and posted
- [ ] Fluids and a real meal, per the diet
- [ ] Someone stays overnight
- [ ] Confirm pharmacy pickup and equipment

Day 2
Settle

- [ ] First full day on the new medication schedule
- [ ] Call to confirm every follow-up on page 6
- [ ] Wound / incision check per instructions
- [ ] Short walk or exercises, if allowed
- [ ] Daily log started: vitals, pain, appetite
- [ ] Help roster filled for the week

Day 3
Watch

- [ ] Compare today's log to yesterday's; note anything worse
- [ ] Bathroom and fluids on track?
- [ ] Pain controlled with what was prescribed?
- [ ] Home health / nurse visit, if ordered
- [ ] Refill anything running low

Day 4
Midweek

- [ ] Caregiver check: did you sleep? Who relieves you?
- [ ] Activity up a notch only if the instructions allow
- [ ] Write questions for the follow-up (page 10)
- [ ] Fridge: still matches the diet?

Day 5
Routine

- [ ] Medications: any side effects to report?
- [ ] Wound / incision check
- [ ] Mood and sleep: note changes
- [ ] Call the office with yellow-list items before the weekend

Day 6
Prepare

- [ ] Gather for the follow-up: this kit, med list, daily log, discharge papers
- [ ] Confirm ride and time
- [ ] Pill organizer refilled for next week

Day 7
Review

- [ ] Read the week's log together: better, same, or worse?
- [ ] Decide what help continues and what can stop
- [ ] Follow-up visit (often within 7 to 14 days)
- [ ] Update the fridge sheet and medication list

DAY 1 TO 2

Follow-ups & services tracker

Everything that was "ordered" or "arranged" at discharge, with a date and a phone number. If a line has no date by day 2, call and get one.

WHAT	DATE & TIME	WHO / WHERE	PHONE	CONFIRMED? RIDE? NOTES
Primary care follow-up Usually within 7 to 14 days				
Specialist / surgeon follow-up				
Home health / visiting nurse First visit date				
Physical / occupational therapy				
Equipment delivery Walker, bed, oxygen, commode				
Lab work or imaging				
Pharmacy pickup / delivery				
Insurance / case manager call				

If something that was "ordered" hasn't called by day 3

Call the hospital's discharge planner or case manager (the number is usually on the discharge summary). Orders get lost between the hospital and the agency more often than anyone admits. Write the call on the log on page 11.

EVERY DAY THIS WEEK · BRING TO THE FOLLOW-UP

Daily vitals & symptom log

Only track what the care team asked for, plus the basics. A week of numbers side by side shows a trend that no single day does.

Week starting: _____ Target ranges from the care team (if given): BP _____ Weight _____ Other _____

	DAY 1	DAY 2	DAY 3	DAY 4	DAY 5	DAY 6	DAY 7
Temperature							
Blood pressure / pulse							
Weight							
Oxygen / breathing							
Pain 0–10							
Ate & drank							
Bathroom							
Sleep							
Mood & alertness							
Wound / incision							
Activity / exercises							
Meds all given?							

Anything new, different, or reported to the care team (date it)

BEFORE THEY COME HOME, OR DAY 1

Setting up the house

Recovery happens in about four places: bed, bathroom, chair, kitchen. Make the path between them short, lit, and clear, and put what they need within arm's reach of each.

Bed

- [ ] On the main floor if stairs are limited
- [ ] Height lets feet touch the floor
- [ ] Lamp, phone, water, glasses within reach
- [ ] Night light to the bathroom
- [ ] Extra pillows for positioning, if instructed
- [ ] Commode or urinal nearby if the bathroom is far

Bathroom

- [ ] Grab bars or a sturdy hold at toilet and shower
- [ ] Shower chair and handheld sprayer if bathing is allowed
- [ ] Non-slip mat; rugs removed
- [ ] Raised toilet seat if standing up is hard

Chair & living space

- [ ] A firm chair with arms, not the low sofa
- [ ] Walker or cane parked where they sit
- [ ] Remote, phone, water, tissues, medications list at hand
- [ ] Cords, rugs, pet beds out of the path
- [ ] Chair to bathroom and chair to kitchen both clear

Kitchen

- [ ] Meals for the week matching the diet, portioned
- [ ] Fluids they'll actually drink
- [ ] Everyday items at counter height
- [ ] Pill organizer on the counter, filled

Equipment

- [ ] Walker / cane adjusted to their height
- [ ] Oxygen set up and explained, tubing routed safely
- [ ] Hospital bed, if ordered, placed and tested
- [ ] Thermometer, BP cuff, scale if the team asked for readings
- [ ] Wound supplies in one bin

Paper

- [ ] "When to call" page posted
- [ ] Medication list on the fridge
- [ ] Discharge papers in one folder
- [ ] This kit on the counter

STILL NEEDED	WHO	BY	DONE

DAY 1 · FILL IN WITH WHOEVER IS HELPING

The help roster

One person can't do seven nights. Write names in boxes, not "we'll figure it out." Friends who ask "what can I do?" get a box.

	DAY 1	DAY 2	DAY 3	DAY 4	DAY 5	DAY 6	DAY 7
Morning Meds, breakfast, wash							
Midday Lunch, exercises, check-in							
Evening Dinner, meds, wound check							
Overnight							
Rides & errands Pharmacy, appointments							
Meals dropped off							

Helpers & how to reach them

NAME	PHONE	GOOD FOR (NIGHTS, RIDES, MEALS, SITTING, DOG)

For the primary caregiver

Your box on this grid matters as much as theirs. If your name is in every row, that's the first problem to fix, before day 3, while people are still offering.

DAY 4 TO 6 · BRING TO THE FIRST VISIT

Questions for the follow-up visit

Check the ones that apply, add your own from the week's log, and write the answers during the visit. Bring this kit, the medication sheet, and the daily log.

Appointment with: _____ Date: _____

About recovery

- [ ] Is this where you'd expect them to be a week out?
- [ ] Here's the daily log. Does anything in it concern you?
- [ ] What should improve next, and by when?
- [ ] What activity is allowed now: stairs, driving, lifting, bathing, being alone?
- [ ] What would make you want to see them sooner?

About medications

- [ ] Here's the changes sheet. Is this list right?
- [ ] Which of the new medications stop, and when?
- [ ] We noticed _____ since starting _____. Related?
- [ ] Can the old prescriptions be cancelled at the pharmacy?

About services

- [ ] Should home health or therapy continue, change, or stop?
- [ ] Is any equipment no longer needed, or newly needed?
- [ ] Who coordinates between you and the specialist?
- [ ] Which release lets you talk with me directly?
- [ ] When is the next visit, and with whom?

Our own questions

Answers & notes from the visit

ONGOING

Call log

Every call with the care team, the pharmacy, the agency, insurance, or the equipment company. Date, who, what was said, what happens next. Ask for a reference number on insurance calls.

DATE	WHO I SPOKE WITH	ABOUT	WHAT WAS SAID / DECIDED	FOLLOW-UP & REFERENCE #

After the first week

If care continues past this week, the companion kit *The First 30 Days* carries on from here: the fridge sheet, the full medication list and weekly log, the documents checklist, and a weekly check-in for the caregiver.

This kit is an organizer for the instructions your care team gives you. It is not medical advice. The warning signs printed here are general; the ones in your discharge papers are specific to this hospital stay and take priority. When in doubt, call the number on page 4.

PRINTABLE KIT · 14 PAGES

FOR ADULT CHILDREN OF AGING PARENTS

The First 30 Days

A caregiver's starter kit for the month after you realize a parent needs your help. One page for each thing that keeps you up at night, in the order you'll need them.

WHAT'S INSIDE

- How to use this kit
- 30-day roadmap, week by week
- Emergency fridge sheet
- Care team & contacts
- Current medication list
- Weekly medication log
- Appointment tracker
- Questions to ask the doctor
- Important documents checklist
- Daily care & symptom log
- Home safety walkthrough
- Weekly caregiver check-in
- Call & decision log

START HERE

How to use this kit

You don't have to do all of this at once. The roadmap on the next page spreads it across four weeks. Print what you need, when you need it.

Print once

- [ ] Emergency fridge sheet
  Fill it in first. Tape it to the fridge and keep a photo on your phone.
- [ ] Care team & contacts
- [ ] Current medication list
  Reprint whenever a medication changes.
- [ ] Important documents checklist
- [ ] Home safety walkthrough
- [ ] 30-day roadmap

Print weekly or as needed

- [ ] Weekly medication log
  One sheet per week. Five copies covers the month.
- [ ] Weekly caregiver check-in
  Sunday night, five minutes, for you.
- [ ] Daily care & symptom log
  Use it when something feels off, or daily if a doctor asked you to watch for changes.
- [ ] Appointment tracker
  Bring it to every visit.
- [ ] Questions to ask the doctor
  Fill it in the night before a visit.
- [ ] Call & decision log

Three habits that make the rest easier

1. Keep one binder. Everything from this kit goes in one place, in this order. When another family member steps in, you hand them the binder, not a phone call.

2. Write down every call. Insurance, pharmacy, the doctor's office. Date, who you spoke to, what they said. The call log at the back exists because you will be asked "who told you that?"

3. Bring the medication list everywhere. Every provider will ask. A current list in your hand is the single most useful thing you can carry into an appointment or an emergency room.

This binder is for

Kept by (primary caregiver)

This kit is an organizer. It helps you record and track information so you can share it accurately with your parent's doctors, pharmacist, and family. It is not medical, legal, or financial advice. For decisions about care, medications, or legal documents, talk with the professionals involved in your parent's care.

THE PLAN

30-day roadmap

Four weeks, one focus each. Check things off as you go. If a week slips, it slips. The order matters more than the pace.

Week 1 · Stabilize

Make sure anyone who walks in the door can keep your parent safe.

- [ ] Fill in the emergency fridge sheet and tape it up
- [ ] Build the current medication list (bottles in hand, not from memory)
- [ ] Fill in care team & contacts
- [ ] Confirm the pharmacy and set up refill reminders
- [ ] Start the weekly medication log
- [ ] Save the primary doctor's after-hours number in your phone

Week 2 · Paperwork

Find out what exists before you need it in a hurry.

- [ ] Work through the important documents checklist
- [ ] Locate insurance cards, Medicare card, photo ID; photograph both sides
- [ ] Ask whether a healthcare power of attorney and advance directive exist
- [ ] Ask the doctor's office which release form lets them talk to you
- [ ] List recurring bills and how each one is paid

Week 3 · Care team & routine

Turn a crisis into a schedule.

- [ ] Book or confirm the next appointment with every current provider
- [ ] Fill in "Questions to ask the doctor" before the first visit
- [ ] Write down the daily routine: meals, meds, sleep, activity
- [ ] Decide who does what among family; write names next to tasks
- [ ] Ask the doctor's office what to watch for and when to call
- [ ] Start the daily care log if anything is being monitored

Week 4 · Home, money, and you

The things that go wrong quietly.

- [ ] Do the home safety walkthrough, room by room
- [ ] Fix the three cheapest, highest-risk items first
- [ ] Set up the call & decision log as your running record
- [ ] Ask what local help exists: Area Agency on Aging, senior center, respite care
- [ ] Do your first weekly caregiver check-in
- [ ] Put one thing on the calendar that is only for you

At the end of 30 days you should have

A fridge sheet anyone can act on · a medication list you trust · the location of every important document · a care team with a next appointment · a home you've walked through with fresh eyes · and a weekly five minutes for yourself.

POST ON THE FRIDGE · KEEP A PHOTO ON YOUR PHONE

Emergency information

In an emergency, call 911 first. Then use this sheet.

Full name		**Date of birth**	
Address			
Preferred language		**Hearing / vision / mobility notes**	

Medical

Diagnoses & conditions			
Allergies (medications, food, latex)			
Current medications Full list in binder			
Blood type (if known)		**Pacemaker / implants / devices**	
Advance directive / DNR on file?		**Where the paperwork is kept**	

Who to call

ROLE	NAME	PHONE
Primary caregiver		
Second contact		
Primary doctor		
Pharmacy		
Preferred hospital		
Insurance (plan & member ID)		

Updated on: _____ Reprint this sheet whenever a medication, doctor, or contact changes.

WEEK 1

Care team & contacts

Everyone involved in your parent's care, on one page. Include the office's after-hours number when they have one.

ROLE	NAME	PHONE	NOTES (PORTAL LOGIN HINT, BEST TIME TO CALL, FAX)
Primary care doctor			
Specialist			
Specialist			
Specialist			
Pharmacy			
Dentist			
Eye doctor / hearing			
Home health / aide agency			
Insurance / Medicare			
Attorney / financial advisor			
Clergy / faith community			
Neighbor with a key			
Family member			
Family member			

WEEK 1 · BRING TO EVERY APPOINTMENT

Current medication list

Fill this in with the bottles in front of you. Include vitamins, supplements, and anything over the counter. Reprint when anything changes.

MEDICATION & STRENGTH	DOSE	WHEN TAKEN	PRESCRIBED BY	WHAT IT'S FOR	REFILL DUE

Allergies & past reactions

Stopped medications (name, date, why)

Pharmacy: _____ Phone: _____ Updated on: _____

WEEKLY · PRINT ONE PER WEEK

Weekly medication log

Write each medication in the left column with its time slot. Initial the box when it's given. A blank box is the fastest way to spot a missed dose.

Week of: _____

MEDICATION · TIME	MON	TUE	WED	THU	FRI	SAT	SUN
Morning							
Midday							
Evening							
Bedtime							
As needed (write dose & time)							

Missed doses, side effects, refills ordered

WEEK 3 · BRING TO EVERY VISIT

Appointment tracker

One row per visit. The last column is the one people forget, and it's the one that matters a week later.

DATE & TIME	PROVIDER	REASON FOR VISIT	WHAT THEY SAID	NEXT STEPS & FOLLOW-UP DATE

Before you leave the office, ask

Is there anything I should watch for before the next visit? · Who do I call if it happens after hours? · Did anything change on the medication list today?

WEEK 3 · FILL IN THE NIGHT BEFORE

Questions to ask the doctor

Check the ones that apply, add your own, and write the answers during the visit. Hand the sheet to the doctor if it's easier than reading it aloud.

Appointment with: _____ Date: _____

About the condition

- [ ] What is the diagnosis, in plain words?
- [ ] What should we expect over the next few months?
- [ ] What changes should we watch for, and which ones mean we call you right away?
- [ ] Is there anything we should stop doing, or start doing, at home?

About medications

- [ ] Here is the full list. Does anything on it concern you?
- [ ] What is each new medication for, and how will we know it's working?
- [ ] What side effects are common, and which ones should we report?
- [ ] Is there anything that should not be taken together, or with food or alcohol?

About care and support

- [ ] Who do we call after hours, and when should we go to the ER instead?
- [ ] Would home health, physical therapy, or other services help? Who orders them?
- [ ] Is it still safe for them to drive / live alone / manage their own medications?
- [ ] Which release form lets you talk with me about their care?
- [ ] When is the next visit, and what should we bring?

Our own questions

Answers & notes from the visit

WEEK 2

Important documents checklist

You are not creating these documents this month. You are finding out which ones exist and where they live. Names vary by state; ask the attorney or clinic for the local equivalent.

DOCUMENT	EXISTS?	WHERE IT'S KEPT	NOTES (WHO HAS COPIES, DATE SIGNED)
Healthcare power of attorney Who speaks for them on medical decisions			
Advance directive / living will Their wishes for end-of-life care			
Medical records release (HIPAA) Lets providers talk to you			
Do-not-resuscitate order (if any) Must be the state's own form to be honored			
Financial power of attorney Who can handle money and bills			
Will or trust			
Photo ID, Social Security card, birth certificate			
Medicare / insurance cards & supplemental policies			
Long-term care insurance policy			
Life insurance policies			
Bank & investment accounts Institution and last 4 digits only			
Pension, retirement, Social Security statements			
Deed, mortgage, or lease			
Vehicle title & insurance			
Military discharge papers (DD-214)			
Marriage / divorce certificates			
Funeral or burial pre-arrangements			
List of online accounts & where passwords are kept Never write passwords on this page			

AS NEEDED · ONE PAGE PER DAY

Daily care & symptom log

Use this when a doctor asked you to watch for something, or when your gut says something is off. Patterns show up on paper long before they show up in memory.

Date: _____ Logged by: _____

CHECK-IN	MORNING	AFTERNOON	EVENING
Mood & alertness			
Ate & drank			
Bathroom			
Mobility & balance			
Sleep / naps			
Vitals, if tracking BP · pulse · temp · glucose · weight			

Pain today (circle)

0 1 2 3 4 5 6 7 8 9 10

Where, and what helped:

Medications given as planned?

☐ Yes ☐ Missed / late (note which):

Anything new or different today

To report to the doctor / to tell the next caregiver

WEEK 4

Home safety walkthrough

Walk through the house the way your parent moves through it: from the bed to the bathroom at night, from the chair to the kitchen. Check what's fine, note what isn't.

Everywhere

- [ ] Loose rugs removed or taped down
- [ ] Clear paths, cords off the floor
- [ ] Night lights bedroom → bathroom
- [ ] Smoke & CO detectors tested
- [ ] Emergency numbers posted (fridge sheet)
- [ ] Phone reachable from bed and chair
- [ ] Thermostat easy to read and reach

Entry & stairs

- [ ] Sturdy handrails on both sides
- [ ] Steps even, edges visible, lit
- [ ] Door locks they can work easily
- [ ] A neighbor or lockbox has a key

Bathroom

- [ ] Grab bars by toilet and in shower
- [ ] Non-slip mat in tub or shower
- [ ] Shower chair or bench if standing is hard
- [ ] Raised toilet seat if needed
- [ ] Water heater set to 120°F or lower
- [ ] Door can be unlocked from outside

Bedroom

- [ ] Lamp reachable from bed
- [ ] Bed height lets feet touch the floor
- [ ] Clear path to the bathroom
- [ ] Glasses, phone, water within reach

Kitchen

- [ ] Everyday items at waist to shoulder height
- [ ] No step stools needed for daily use
- [ ] Stove knobs clearly marked; auto-off if forgetful
- [ ] Fire extinguisher within reach
- [ ] Expired food and medicine cleared out

Medications

- [ ] One storage place, not scattered
- [ ] Weekly pill organizer if more than 3 meds
- [ ] Old prescriptions removed
- [ ] List on the fridge matches the bottles

WHAT NEEDS FIXING	WHO WILL DO IT	BY WHEN	DONE

WEEKLY · SUNDAY NIGHT · FIVE MINUTES

Weekly caregiver check-in

This page is about you. Caregivers who burn out usually saw it coming on paper weeks before they felt it. This is the paper.

Week of: _____

This week I slept

☐ Enough ☐ Not quite ☐ Badly, most nights

My own health

☐ Kept my appointments ☐ Skipped or postponed one ☐ Haven't scheduled

What went well

What was hardest

One task I will hand to someone else next week

Task, and who I'll ask:

One thing I will do that is only for me

On the calendar, day and time:

Warning signs worth taking seriously

- ☐ Snapping at my parent or family
- ☐ Skipping meals or living on coffee
- ☐ Not sleeping even when I can
- ☐ Dreading the phone ringing
- ☐ Nobody else knows the routine
- ☐ Haven't seen a friend in weeks

Three or more checked for two weeks running: that is the moment to call someone. Your doctor, a caregiver support line, your Area Agency on Aging, or the person on the contacts page you trust most.

ONGOING · THE RUNNING RECORD

Call & decision log

Every call with insurance, the pharmacy, a doctor's office, or a family decision. Date, who, what was said, what happens next. Reprint as the pages fill.

DATE	WHO I SPOKE WITH Name & organization	ABOUT	WHAT WAS SAID / DECIDED	FOLLOW-UP & REFERENCE #

Tip

Ask for a reference or case number on every insurance call and write it in the last column. It turns "someone told me" into something the next person can look up.